S5 5 Support working conditions administration and governance

5 支援工作條件行政及管治

Sources / 來源: R1 support and governance; R2 §5; A05–A29 | Open actions / 待辦: G06 G18 G19 G20 G21 G22

5.1 Oversight and accountability / 5.1 監督及責任

Proposed Joint Programme Board: SJTU academic lead, HKIO operational lead, assessment lead, clinical governance lead, QA lead and trainee representative. Confirm names, authority, quorum, meeting frequency, escalation route, delegated decisions and succession cover. Keep minutes, decision records and a controlled evidence register. Agreements must define delivery, award, clinical liability, data responsibilities and financial commitments.

建議聯合課程委員會包括上海交大學術主管、HKIO 營運主管、評核主管、臨床管治主管、質素主管及學員代表。須確認姓名、權限、法定人數、會議頻率、上報途徑、授權及替補。保存會議紀錄、決策及受控證據目錄；合作協議應界定教學、授證、臨床責任、資料及財務承諾。

5.2 Safety and local policies / 5.2 安全及本地政策

Appendices A05–A25 retain the reference numbering and supply adaptation specifications, draft workflows and forms. None establishes adoption of Maltese legislation, GDPR applicability or Mater Dei procedures. Each clinical site must supply its approved local policies, accountable officers, training records and implementation evidence. Specific additions cover safeguarding, medicines, candour, appeals, malpractice, fitness to practise, welfare and reasonable adjustments (A26–A29).

附錄 A05–A25 沿用參考編號，提供本地化要求、流程草稿及表格，不代表採用馬耳他法例、確認 GDPR 適用或採納 Mater Dei 程序。每個臨床地點須提供本地批准政策、責任人、培訓及執行證據。另設 A26–A29 涵蓋保障弱勢、藥物、坦誠披露、上訴、失當、適任執業、福利及合理調適。

5.3 Student support and continuity / 5.3 學員支援及持續照護

Provide a named academic adviser, confidential pastoral contact, IT support and a route independent of the supervisor for concerns. Proposed underperformance response: identify evidence, discuss with trainee, agree supervised remediation and review date, record reassessment and appeal rights. Clinical restrictions must be based on patient safety. Patient handover and retention follow-up must continue during absence or withdrawal. Contact names and service response standards are [MISSING].

應提供學術顧問、保密福利聯絡人、資訊科技支援及獨立於督導者的關注渠道。建議處理表現不足：確認證據、與學員討論、協定受督導補救及覆核日期、記錄重評與上訴權。臨床限制應以病人安全為依據；缺席或退學期間亦須持續交接及保持期跟進。聯絡人及服務回應標準[待補]。